Male circumcision: Who should decide?

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In a recent editorial, Dr. Andrew L. Freedman, a member of the 2012 AAP task-force on circumcision, argues that "health issues" are only a "small piece of the puzzle" when it comes to this procedure. "Although parents may use the conflicting medical literature to buttress their own beliefs and desires," he writes, "for the most part parents choose what they want for a wide variety of nonmedical reasons," including "religion, culture, aesthetic preference, familial identity, and personal experience."[1, p. e20160594] Dr. Freedman is correct that nonmedical factors may reasonably factor into a person's decision about circumcision. But one may question his assumption that this person should be someone other than the individual who would be affected by the surgery were it to take place.

In the *Journal of Medical Ethics*, Akim McMath notes: "People disagree over what constitutes a harm and what constitutes a benefit" when it comes to circumcision.[2, p. 689] For example, "Some people believe circumcision benefits the child by bringing him closer to God, while others disagree. In light of such disagreement, some commentators conclude that the parents

should decide." But this does not follow. After all, "the child will have an interest in living according to his own values, which may not reflect those of his parents … Only the child himself, when he is older, can be certain of his values." Thus, "if disagreement over values constitutes a reason to let the parents decide, it constitutes an even stronger reason to postpone the decision until the child himself can decide."[2, p. 689]

Against this view, it is sometimes argued that circumcision is "less risky" in infancy, such that deferring the decision to an age-of-consent would be medically inadvisable. But this is to confuse several issues. First, the claim that infant circumcision carries less risk compared to adult circumcision is based largely upon appeals to retrospective comparisons of different studies (or data sets), with results drawn from dissimilar populations. Accordingly, these comparisons do not typically adequately control for such variables as the skill of the practitioner, the specific technique employed, the setting of the surgery, and so on.[3]

But even granting an increase in the relative risk of complications between the surgery performed in childhood vs. adulthood, one could argue that it is the difference in absolute risk that is most ethically relevant. Even proponents of circumcision agree that the absolute likelihood of clinically significant, difficult-to-resolve surgical complications associated with circumcision is low, irrespective of the age at which the procedure is performed. Given such a low baseline risk, the existence of a relative-risk reduction in the incidence of adverse events in infancy compared to adulthood is unlikely to be morally decisive.[4]

Consistent with this view, the CDC states: "Delaying male circumcision until adolescence or adulthood obviates concerns about violation of autonomy."[5, p. 39] Therefore, any medical disadvantages associated with such a deferral "would be ethically compensated to some extent by the respect for the [bodily] integrity and autonomy of the individual."[5, pp. 39–40]

References

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